

TIMEPIE LONGEVITY FORUM · BIOHACKER TRACK · SHANGHAI 2026



# BIOHACK.IT

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HACKING BIOLOGY

An open laboratory for longevity.

Turning thousands of individual self-experiments into structured, comparable, open data — with safety engineered in, not bolted on.

Open source · AGPL-3.0 · Non-profit — Hacking Biology

## THE GAP

# Longevity intervention is running far ahead of longevity measurement.

People already adopt off-label drugs, peptides, HBOT, plasmapheresis. But the protocol lives in spreadsheets, forum posts and chat prompts — not comparable, not followable, not readable by a physician.

130+

biomarkers a serious protocol tracks —  
blood, DNA methylation, DEXA

$N = 1$

an isolated self-experiment carries almost  
no statistical power

0

shared representation layer for protocol &  
biomarker across people and labs

Bryan Johnson has a lab and a team. Everyone else has a spreadsheet. A thousand N-of-1 on the same protocol, made comparable, are a different scientific object — and it doesn't exist yet.

## WHO IS EXPOSED

The beginner who copies is the person most at risk.

### PRIMARY USER

#### **The experienced biohacker**

Designs, self-administers, iterates — but the results today are just anecdotes: claimed, not measured, impossible to compare.

### HIGHEST RISK

#### **The beginner who copies**

Reads a forum thread or an influencer hyping a compound and starts taking it — no baseline, no safety markers, no measurement.

### RECIPIENT, NOT USER

#### **The treating physician**

Today receives, at best, a handwritten sheet. Deserves a legible, complete view of what is taken and why.

The person who copies an off-label compound off a forum thread — without knowing to monitor kidney and liver — is exactly the case where the platform protects health.

## THE MECHANISM

# Follow and copy the people who get results — and see their biomarkers.

The social dynamic that powers copy-trading in finance, applied to health. The value is not the single datapoint; it is the ability to follow, compare and copy, anchored to structured, public biomarkers rather than anecdote.

- ◆ **A protocol becomes something you can subscribe to** — and it carries its safety markers with it.
- ◆ **Every compound and intervention gets a thread** anchored to real, comparable data.
- ◆ **A living community is the only quality control** that actually works on self-reported data.

## THE PLATFORM

# Document your protocol. Measure it. Share it.

### 01 — DOCUMENT

#### What you take & do

Substances, therapies, exercise, nutrition — with dose, timing and real cycle schemas. Create from scratch, or copy any public protocol with one click.

### 02 — MEASURE

#### Efficacy & safety

Biomarkers with reference, optimal and safety ranges, plus biological clocks (PhenoAge, epigenetic, glycation). Upload past lab reports, get your history in graphs.

### 03 — SHARE

#### Open by default

Publish your protocol and its outcomes. Follow and copy the people who get results, seeing their public biomarkers — the community is the quality control.

Open source, free, non-profit — built on the community that already exists, not a forum rebuilt from scratch.

## DESIGN PRINCIPLES

# Rigor is a design decision, not a disclaimer.

- ◆ **Describes, never prescribes.** Suggests what to measure, never what to take or how much.
- ◆ **Every claim graded to its evidence.** Human clinical → observational → animal → in vitro → anecdotal.
- ◆ **The LLM writes words, never numbers.** Extraction, mapping and thresholds are deterministic and reproducible.
- ◆ **It fails loudly.** Never drops an item silently — it shows the diff between input and output.
- ◆ **It stops instead of guessing.** When data is insufficient, it says so — no approximate index.
- ◆ **Sensitive data, heightened consent.** The most identifying data is shared only behind an explicit, informed gate.

## DATA MODEL

# Three questions, deliberately kept apart.

### TestingProtocol

What I measure, and when. A versioned set of measures and timepoints.

### TreatmentPlan

What I take and do. Versioned interventions — self-managed or physician-assigned.

### Goal

What I want to change. Mapped onto the Hallmarks of Aging.

Substance → SafetyRule → Biomarker → Measurement → Dashboard

*“What you take” and “how you are” are the same data structure, read from two sides.*

## ARCHITECTURE

# One platform, one model.

24 capabilities, built in the open and grown by community review. Six families:

### Protocols

create · copy · import · versioned

### Blood Layer

biomarkers · clocks · deterministic

### Safety guardrails

baseline · inheritance · dose check

### Community & Studies

follow/copy · cohorts · N-of-1

### Open Data & Standards

LOINC/UCUM · FHIR/OMOP · clonable

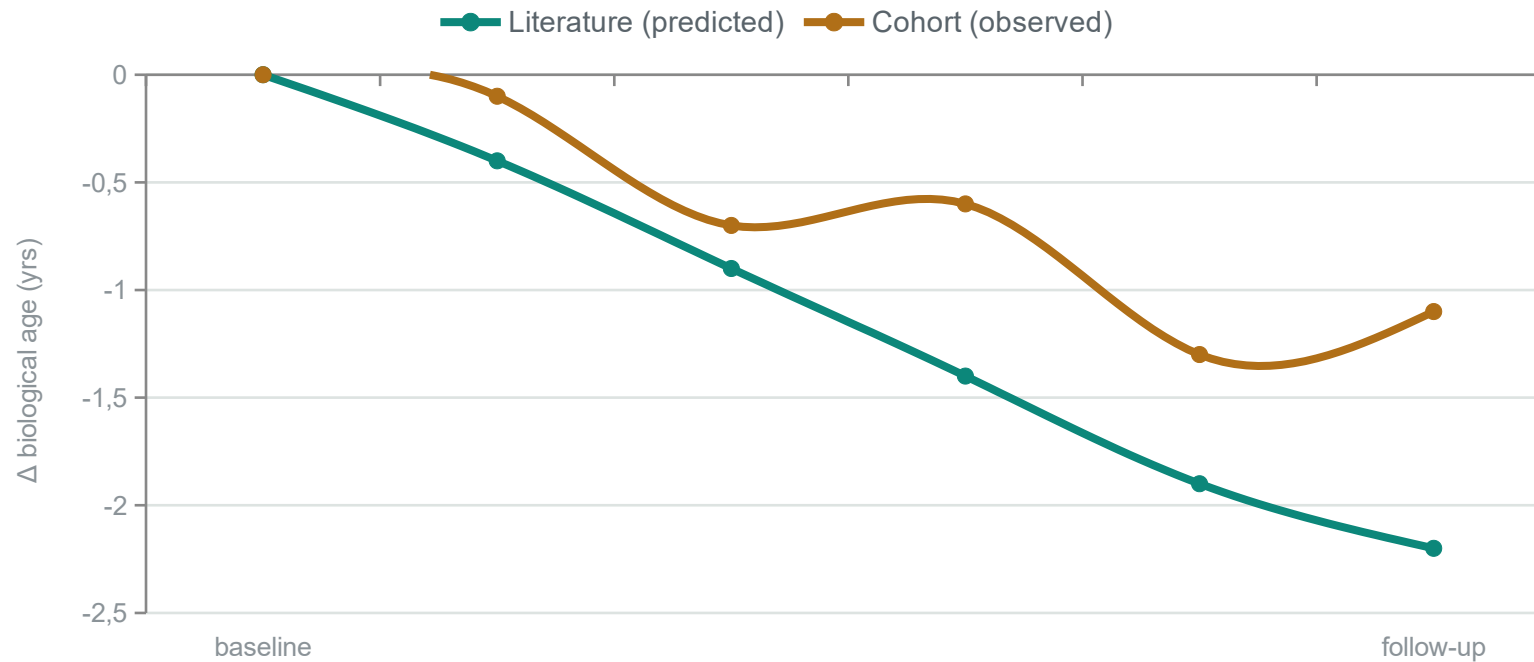
### AI, Evidence & Provenance

AI4L · Evipedia · attribution

The Blood Layer and the Daily Log ship first — useful to a single person, with no community required.

## WHAT NO ONE ELSE CAN BUILD

Literature predicts -2.2 years. The 340 who did it show this.



### THE ORIGINALITY

**Outcome ↔  
Biomarker**

Anyone can draw the first line — it is in the literature. Only a platform with a living community and shared, standards-coded biomarkers can draw the second, and put them side by side.

Illustrative. The link between an outcome and its biomarker is what makes the two columns comparable.

## HARM REDUCTION

# Follow a protocol — inherit its safety.

- ◆ **Mandatory baseline** before you can follow a protocol at all — plus an explicit risk acknowledgment.
- ◆ **Safety markers inherited automatically** from every active compound — safety is never opt-in.
- ◆ **Dose sanity check** catches the extra zero — plausibility validation of a number, not clinical advice.
- ◆ **Overdue is a safety gap** “rapamycin active for 94 days, liver panel overdue by 34.”

This is the module that justifies the project. Biohackers already take the risk today — without any of it.

## PHASE 1 · THE PRODUCT YOU CAN DESCRIBE IN PUBLIC

Upload your past lab reports. Get your clinical history in graphs. Free.

### A HARD BOUNDARY

#### **The LLM writes words, never numbers**

Extraction, mapping and thresholds are deterministic and reproducible. The model only produces the plain-language narration — generated once and cached.

### BUILT FOR AGGREGATION

#### **LOINC + UCUM + method, every value**

The only architecture compatible with releasing open data. Three ranges per analyte: laboratory reference, longevity-optimal, safety.

A sentence you can say on television without pronouncing the word “biohacking”, with no regulatory exposure — and it works with a single user and no community.

**BUILT FOR HACKERS · OPEN BY DEFAULT**

# Share your protocol. Share your outcomes. In public.

## **Public is the point**

Server-hosted, always on, public by default. Your protocol and its biomarker outcomes are meant to be seen, followed and copied — how the community learns faster than any of us alone.

## **Your data becomes evidence**

Every value you share, standards-coded, joins an open dataset on interventions no trial will ever fund. Contributing is the invitation.

## **Agent Access · MCP**

A read-only token so you — or anyone you allow — can query the data with an agent: Claude, Cursor, a Nostr bot. No competitor will ship this.

Walled gardens are impossible by construction: AGPL means anyone running it as a service must open the source. Fork it.

**BUILD NOTHING THAT ALREADY EXISTS**

# Standards, not silos.

## **Speak the standards**

HL7 FHIR internally; OMOP CDM export for research — so what you produce is real research data with proper coding, not another wellness silo.

## **Stand on open shoulders**

Evidence from Evipedia (Forever Healthy, CC BY 4.0); AI reviews via the AI4L audit framework; component reuse from get-based — all attributed, notified, contributed back.

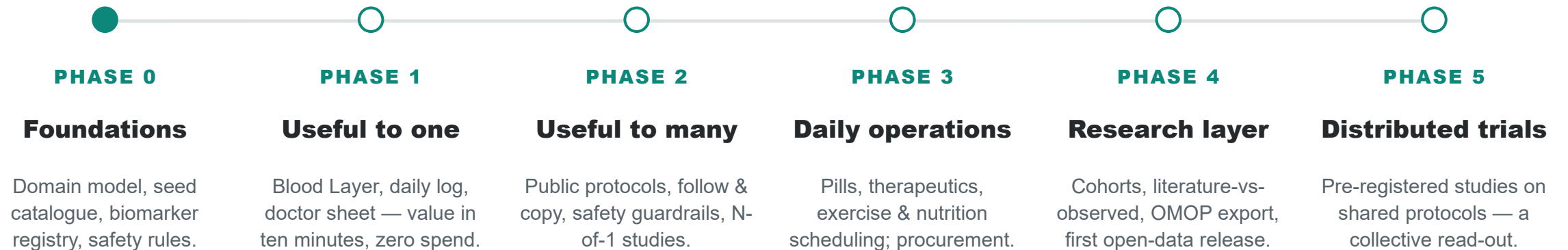
## **Existing community**

The forum is not rebuilt: Rapamycin News is already there. Link first, cohorts second — never the reverse.

Open data released only with cohort thresholds and declared provenance. A small clean dataset beats a large uncomparable one.

## FROM ONE PERSON TO A DISTRIBUTED TRIAL

An open, grassroots, distributed longevity trial — producing data.



## THE INVITATION



Everyone buys anti-aging. **Almost no one measures it.**

Let's measure it — together, in the open.

Open infrastructure and open data for the longevity community — fork it, run it local, own your data.  
The community already exists; now it gets its instrument.

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